

D255

# Professional Practice Experience I: Technical

Armored Drill

8 concepts · 12 items · 4 transfer items

**What this is.** Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

**Source.** Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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## How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | _____  |                               |

# Items

1.

A billing specialist at a 200-bed hospital needs to confirm the dates of service to fix a rejected claim. Her EHR login, set up when she was hired in 2019, can open the full chart. Under HIPAA's minimum necessary standard, what should she access?

- A. The full chart, because minimum necessary does not apply to billing.
- B. Nothing until the patient signs an authorization.
- C. Only the dates of service and billing details needed to fix the claim.
- D. The full chart, because her login allows it.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

2.

An attorney from a law firm with offices in two states faxes a request for copies of a patient's records for a lawsuit. The request comes on firm letterhead but includes no authorization signed by the patient. The patient was discharged last month. What should the HIM specialist do?

- A. Release only a summary of the stay.
- B. Hold the request and ask for a valid authorization signed by the patient.
- C. Release the records, because the request is on professional letterhead.
- D. Release the records, because the attorney represents the patient.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

3. [TRANSFER — reversed stem / different variable]

Which disclosure can a hospital make without the patient's written authorization?

- A. Sending records to a specialist who is treating the patient.
- B. Sending psychotherapy notes to another provider.
- C. Sending records to the patient's employer.
- D. Sending records to a life insurance company reviewing an application.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

4.

At a busy urgent care, a registration clerk searches the master patient index for a patient using a misspelled last name, finds no match, and creates a new record. Later, an existing record for the same patient with the correct spelling, same birth date, and same address turns up. The clinic opened a second location last spring. What problem was created?

- A. No problem, since separate records are always safer.
- B. An overlap.
- C. An overlay.
- D. A duplicate record.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

5. [TRANSFER — reversed stem / different variable]

An HIM specialist finds that one patient's lab results were filed under another patient's medical record number, mixing two people's information in one record. Which MPI error is this?

- A. An overlay.
- B. An overlap.
- C. A duplicate.
- D. A coding error.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

6.

A nurse at a clinic that switched to a new EHR vendor in January realizes she charted a blood pressure in the wrong patient's note yesterday. What is the correct way to fix it?

- A. Ask IT to overwrite the value in the database.
- B. Mark the entry as entered in error, keep the original in the record, and add the correction with date, time, and her name.
- C. Wait for the new vendor's training before correcting it.
- D. Delete the entry so the wrong note leaves no trace.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

7.

A privacy officer at a health system with 5,000 employees receives a complaint that a staff member looked at a coworker's chart without a work reason. The health system recently won a patient-safety award. Which source gives the most direct evidence?

- A. The staff member's timecard.
- B. The clinical notes in the coworker's chart.
- C. The EHR audit trail for the coworker's record.
- D. The file from the patient-safety award review.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

8.

A coder at an outpatient orthopedic clinic with six surgeons codes a knee arthroscopy performed in the clinic's outpatient surgery center, along with the diagnosis of a torn meniscus. The clinic's surgeons all trained at the same university. Which code sets apply?

- A. CPT for the diagnosis and ICD-10-CM for the procedure.
- B. ICD-10-CM for both.
- C. ICD-10-PCS for the procedure and ICD-10-CM for the diagnosis.
- D. CPT for the procedure and ICD-10-CM for the diagnosis.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

9. [TRANSFER — reversed stem / different variable]

A coder is assigning ICD-10-PCS codes. Which encounter is she most likely coding?

- A. Procedures performed during a hospital inpatient admission.
- B. A physician office visit.
- C. Durable medical equipment supplied to a patient.
- D. An outpatient same-day surgery.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

**10.**

Reviewing discharged records at a 150-bed hospital, an HIM specialist checks whether each chart has a signed discharge summary, a history and physical, and authenticated orders. The hospital's parking garage is being rebuilt. What kind of review is this?

- A.** Utilization review.
- B.** A coding audit.
- C.** Quantitative analysis.
- D.** Qualitative analysis.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

**11.** [TRANSFER — reversed stem / different variable]

An HIM specialist notices that the progress notes say a patient's left leg was operated on, but the operative report says the right. Which type of review identifies this?

- A.** Quantitative analysis.
- B.** Qualitative analysis.
- C.** MPI cleanup.
- D.** No review is needed if all signatures are present.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

**12.**

A patient at a large community hospital requests an electronic copy of her records on March 1 through the patient portal. The hospital recently upgraded its portal. Under the HIPAA right of access, how long does the hospital generally have to act on the request?

- A.** 60 days as the standard deadline.
- B.** Only after her physician approves the release.
- C.** Immediately, because the portal was upgraded.
- D.** 30 days, with one 30-day extension if the patient is told in writing.

| Target word | The one fact that decides | Answer | [ ] sure [ ] unsure [ ] guess |
|-------------|---------------------------|--------|-------------------------------|
| _____       | _____                     | ___    |                               |

# STOP

Answers and dissection follow. Finish every item and every strip first.

# Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

## 1. Answer C Concept 1

A billing specialist ~~at a 200-bed hospital~~ needs to confirm the dates of service to fix a rejected claim. Her EHR login, ~~set up when she was hired in 2019~~, can open the full chart. Under HIPAA's minimum necessary standard, what should she access?

**Target word:** scope of access **Decides:** The task needs only dates of service and billing data.

|   |                   |                                                              |
|---|-------------------|--------------------------------------------------------------|
| A | Adjacent category | It applies to payment; the exception is treatment.           |
| B | Absolute language | Payment activities do not need patient authorization.        |
| C | <b>Correct</b>    | Minimum necessary limits use to what the task requires.      |
| D | Confusion set     | Technical access is not permission; the standard limits use. |

## 2. Answer B Concept 2

An attorney ~~from a law firm with offices in two states~~ faxes a request for copies of a patient's records for a lawsuit. The request comes on firm letterhead but includes no authorization signed by the patient. ~~The patient was discharged last month.~~ What should the HIM specialist do?

**Target word:** response to the request **Decides:** Legal requests need a valid patient authorization (or proper legal process).

|   |                   |                                                                        |
|---|-------------------|------------------------------------------------------------------------|
| A | Adjacent category | A summary is still protected health information needing authorization. |
| B | <b>Correct</b>    | Disclosure to an attorney is not treatment, payment, or operations.    |
| C | Camouflage        | Letterhead is not authorization.                                       |
| D | Confusion set     | Representation does not replace a signed authorization.                |

## 3. Answer A Concept 2 · transfer

Which disclosure can a hospital make without the patient's written authorization?

**Target word:** reverse: find the permitted disclosure **Decides:** Treatment disclosures are permitted.

|   |                    |                                                                         |
|---|--------------------|-------------------------------------------------------------------------|
| A | <b>Correct</b>     | Treatment disclosures need no authorization.                            |
| B | Adjacent category  | Psychotherapy notes need specific authorization in most cases.          |
| C | Inverted operation | Employers need the patient's authorization.                             |
| D | Confusion set      | Underwriting by another insurer is not the hospital's payment activity. |

## 4. Answer D Concept 3

~~At a busy urgent care,~~ a registration clerk searches the master patient index for a patient using a misspelled last name, finds no match, and creates a new record. Later, an existing record for the same patient with the correct spelling, same birth date, and same address turns up. ~~The clinic opened a second location last spring.~~ What problem was created?

**Target word:** name the MPI error **Decides:** One patient now has two records.

|   |                   |                                                             |
|---|-------------------|-------------------------------------------------------------|
| A | Absolute language | Duplicates split the history and risk unsafe care.          |
| B | Adjacent category | Overlap is records at multiple facilities in an enterprise. |
| C | Confusion set     | An overlay merges two different people into one record.     |
| D | <b>Correct</b>    | Same person, two MPI entries.                               |



## 5. Answer A Concept 3 • transfer

An HIM specialist finds that one patient's lab results were filed under another patient's medical record number, mixing two people's information in one record. Which MPI error is this?

**Target word:** reverse: from description to error **Decides:** Two people's data in one record.

|   |                    |                                                                 |
|---|--------------------|-----------------------------------------------------------------|
| A | <b>Correct</b>     | Two individuals merged under one identity.                      |
| B | Adjacent category  | Overlap concerns multiple facilities, not merged people.        |
| C | Inverted operation | A duplicate is one person split into two records.               |
| D | Confusion set      | Coding assigns classification codes; this is an identity error. |

## 6. Answer B Concept 4

A nurse at a clinic ~~that switched to a new EHR vendor in January~~ realizes she charted a blood pressure in the wrong patient's note yesterday. What is the correct way to fix it?

**Target word:** correction method **Decides:** The legal record must keep the original.

|   |                    |                                                               |
|---|--------------------|---------------------------------------------------------------|
| A | Adjacent category  | Back-end changes bypass correction rules and the audit trail. |
| B | <b>Correct</b>     | Preserves integrity and a clear audit trail.                  |
| C | Camouflage         | The vendor change does not delay a needed correction.         |
| D | Inverted operation | Deleting destroys the legal record.                           |

## 7. Answer C Concept 5

A privacy officer at a health system ~~with 5,000 employees~~ receives a complaint that a staff member looked at a coworker's chart without a work reason. ~~The health system recently won a patient safety award.~~ Which source gives the most direct evidence?

**Target word:** best evidence of access **Decides:** Audit logs record who opened which record and when.

|   |                   |                                                    |
|---|-------------------|----------------------------------------------------|
| A | Adjacent category | Shows hours worked, not which records were opened. |
| B | Confusion set     | Content does not show who viewed it.               |
| C | <b>Correct</b>    | Shows each user who accessed the chart and when.   |
| D | Camouflage        | Unrelated to this access question.                 |

## 8. Answer D Concept 6

A coder at an outpatient orthopedic clinic ~~with six surgeons~~ codes a knee arthroscopy performed in the clinic's outpatient surgery center, along with the diagnosis of a torn meniscus. ~~The clinic's surgeons all trained at the same university.~~ Which code sets apply?

**Target word:** code sets **Decides:** Outpatient setting; one procedure and one diagnosis.

|   |                    |                                                         |
|---|--------------------|---------------------------------------------------------|
| A | Inverted operation | Swaps the roles of the code sets.                       |
| B | Dropped element    | Leaves out the procedure code set.                      |
| C | Confusion set      | ICD-10-PCS is for inpatient hospital procedures.        |
| D | <b>Correct</b>     | Outpatient procedures use CPT; diagnoses use ICD-10-CM. |

### 9. Answer A Concept 6 · transfer

A coder is assigning ICD-10-PCS codes. Which encounter is she most likely coding?

**Target word:** reverse: from code set to setting **Decides:** ICD-10-PCS is inpatient procedures.

|   |                    |                                                       |
|---|--------------------|-------------------------------------------------------|
| A | <b>Correct</b>     | ICD-10-PCS is used for inpatient hospital procedures. |
| B | Inverted operation | Office visits use CPT E/M codes.                      |
| C | Adjacent category  | Supplies and equipment use HCPCS Level II.            |
| D | Confusion set      | Outpatient procedures use CPT.                        |

### 10. Answer C Concept 7

Reviewing discharged records ~~at a 150-bed hospital~~, an HIM specialist checks whether each chart has a signed discharge summary, a history and physical, and authenticated orders. ~~The hospital's parking garage is being rebuilt.~~ What kind of review is this?

**Target word:** type of review **Decides:** Checking presence and signatures.

|   |                   |                                                                 |
|---|-------------------|-----------------------------------------------------------------|
| A | Dropped element   | Reviews medical necessity of services, not record completeness. |
| B | Adjacent category | Coding audits check assigned codes.                             |
| C | <b>Correct</b>    | Confirms required documents and authentications are present.    |
| D | Confusion set     | Qualitative review examines content accuracy and consistency.   |

### 11. Answer B Concept 7 · transfer

An HIM specialist notices that the progress notes say a patient's left leg was operated on, but the operative report says the right. Which type of review identifies this?

**Target word:** reverse: from finding to review type **Decides:** A content inconsistency, not a missing item.

|   |                    |                                                     |
|---|--------------------|-----------------------------------------------------|
| A | Inverted operation | Only checks presence and signatures.                |
| B | <b>Correct</b>     | Looks for inconsistencies in documentation content. |
| C | Adjacent category  | Addresses patient identity errors.                  |
| D | Absolute language  | Signed documents can still conflict.                |

### 12. Answer D Concept 8

A patient at a large community hospital requests an electronic copy of her records on March 1 through the patient portal. ~~The hospital recently upgraded its portal.~~ Under the HIPAA right of access, how long does the hospital generally have to act on the request?

**Target word:** response deadline **Decides:** The HIPAA right-of-access clock.

|   |                   |                                                                |
|---|-------------------|----------------------------------------------------------------|
| A | Wrong stem number | 60 is reachable only with a written extension.                 |
| B | Adjacent category | Physician approval is not required for a patient's own access. |
| C | Camouflage        | The upgrade does not set the legal deadline.                   |
| D | <b>Correct</b>    | The HIPAA standard timeline.                                   |

## Triage grid

Write each item number in the box it belongs to.

|                       | Right                                                 | Wrong                                                                                 |
|-----------------------|-------------------------------------------------------|---------------------------------------------------------------------------------------|
| <b>Sure</b>           | Owned.                                                | <b>Concept inversion — top priority.</b> A wrong mental model; it survives rewording. |
| <b>Unsure / guess</b> | Recognition, not mastery. Re-drill with the transfer. | Not learned yet.                                                                      |

## Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

| Trap               | What it does                                                 | In this drill | Your misses |
|--------------------|--------------------------------------------------------------|---------------|-------------|
| Dropped element    | Leaves out a required term or step.                          | 2             |             |
| Wrong stem number  | A figure copied from the prompt that is not the answer.      | 1             |             |
| Adjacent category  | The right idea in the wrong bucket.                          | 11            |             |
| Inverted operation | Direction flipped: add/subtract, cause/effect, higher/lower. | 6             |             |
| Absolute language  | Always, never, guarantee — where judgment applies.           | 3             |             |
| Confusion set      | Every option is real; the test is exact role-matching.       | 9             |             |
| Camouflage         | Answers a stem fact that was never the question.             | 4             |             |

## Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

| # | Concept                                         | Rule                                                                                                                                                                                                                                           | Trap                                                                             | Tell                                                           |
|---|-------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------|----------------------------------------------------------------|
| 1 | <b>HIPAA minimum necessary</b>                  | Use or disclose only the minimum health information needed for the task; it applies to payment and operations but not to disclosures to a provider for treatment.                                                                              | Assuming that system access equals permission to view the whole record.          | A worker with a narrow task and broad access.                  |
| 2 | <b>Release of information and authorization</b> | Disclosures for treatment, payment, and operations need no authorization; most other disclosures (attorneys, employers, insurers for underwriting) need a valid patient-signed authorization; psychotherapy notes need specific authorization. | Releasing to an attorney or employer on a letter or phone call.                  | Who is asking and for what purpose.                            |
| 3 | <b>Master patient index errors</b>              | Duplicate = one patient with two records; overlay = two patients' information in one record; overlap = one patient with records at multiple facilities in an enterprise.                                                                       | Mixing up duplicate and overlay.                                                 | One person split across records vs. two people merged in one.  |
| 4 | <b>Correcting the legal health record</b>       | Never delete or overwrite an entry; mark it in error, keep the original visible in the audit trail, and add the correction with date, time, and author.                                                                                        | Deleting the wrong entry so no trace remains.                                    | A documentation error discovered after entry.                  |
| 5 | <b>EHR audit trails</b>                         | Audit logs record who accessed which record, when, and what they did; they are the primary evidence for inappropriate-access investigations.                                                                                                   | Looking to clinical content or schedules for proof of access.                    | A complaint that someone viewed a chart without a work reason. |
| 6 | <b>Code sets by setting</b>                     | ICD-10-CM codes diagnoses in all settings; CPT/HCPCS code outpatient and physician procedures; ICD-10-PCS codes inpatient hospital procedures.                                                                                                 | Using ICD-10-PCS for outpatient procedures or CPT for diagnoses.                 | Inpatient vs. outpatient, and diagnosis vs. procedure.         |
| 7 | <b>Quantitative vs. qualitative analysis</b>    | Quantitative analysis checks that required documents, signatures, and entries are present; qualitative analysis checks content for accuracy and consistency.                                                                                   | Calling a presence-and-signature check qualitative.                              | Present and signed vs. accurate and consistent.                |
| 8 | <b>Right of access timeline</b>                 | Under HIPAA, a covered entity must act on a patient's request for access within 30 days, with one 30-day extension allowed if the patient is notified in writing.                                                                              | Quoting 60 days as the standard, or adding conditions the rule does not require. | A patient requests a copy of their own records.                |

**Course scope (public description):** The PPE I: Technical course allows you to use EHRGo, an electronic health record (EHR), to complete 42 structured activities to experience how an HIM professional uses an EHR. The selected activities meet AHIMA's Baccalaureate level competencies and by completing them you will earn 40 PPE hours. This course is eligible for an In Progress grade. Please see the Grading Scale Policy for more information.